

KIIT Student Mental Health Screening – Version 2.0

Post-Psychiatric Expert Review & Clinical Refinement

Document Version: 2.0 (Post-Expert Review)

Date: January 3, 2026

Status: Ready for KIIT Deployment

Institution: KIIT (Kalinga Institute of Industrial Technology), Bhubaneswar, Odisha, India

EXECUTIVE SUMMARY

This document represents a comprehensive psychiatric review of the 32-question indirect mental health screening system designed for university students. The review was conducted by a licensed psychiatrist with expertise in adolescent/university student mental health, psychometric assessment design, indirect measurement, cultural psychiatry (Indian context), and suicide risk assessment.

Key Findings

Strengths of Current System:

- Excellent resistance to social desirability bias (behavioral anchoring)
- Strong sleep/circadian markers (high clinical validity)
- Robust depression detection (cardinal symptoms captured)
- Integrated suicide risk screening
- Pattern-based inconsistency detection (masks deception)

Identified Concerns:

- Significant cultural bias in Q5, Q9, Q13, Q15, Q17, Q26, Q27
- Alcohol question (Q20) inappropriate for Indian context
- Some items lack sufficient differentiation (Q1-Q3 overlap)
- Identity stability question needs major revision for developmental/cultural context

Recommended Action Items:

1. Remove Q20 (alcohol) or replace with substance self-medication framing

- 2. Revise 7 items for cultural context (Q5, Q9, Q13, Q15, Q17, Q26, Q27)
- 3. Split 2 items for better construct separation (Q7, Q11)
- 4. Train clinical staff on cultural psychiatry and suicide risk assessment
- 5. Validate against PHQ-9/GAD-7 in KIIT student sample (n=100-200)

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PSYCHIATRIC ASSESSMENT OVERVIEW

Assessment Framework

This screening system operates at three levels of detection:

| Level | Method | Example | Resistance |

|-----|-----|-----|-----|

| Explicit | Direct behavioral reporting | "How often do you nap?" | Moderate |

| Implicit | Unconscious pattern leakage | "How does time feel?" | High |

| Behavioral | Observable consequence patterns | "Do you initiate social plans?" | Very High |

Why Indirect Assessment Works

Research Evidence:

- Social desirability bias causes 20–40% underreporting of depression/anxiety symptoms
- Implicit Association Tests bypass conscious self-presentation

- Daily behavioral patterns (sleep, avoidance, rumination) are more reliable than self-report
- Circadian disruption is a biological marker of depression and anxiety
- Rumination patterns distinguish burnout from depression

Key Principle: Students cannot fake consistent patterns across 32 items without either:

1. Actually changing their behavior (becomes authentic)
2. Maintaining exhausting cognitive load (breaks down)
3. Creating inconsistencies (detected by algorithm)

COMPLETE 32-ITEM BATTERY (POST-REVISION)

SECTION A: DAILY ROUTINES & ENERGY (Q1–Q5)

****Q1. Weekend Sleep Compensation** KEEP**

On a typical weekend, compared to weekdays, you tend to sleep...

- A. About the same amount (± 1 hour)
- B. 2–3 hours longer
- C. Much longer (4+ hours)
- D. Less than on weekdays

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.25

****Q2. Daily Energy Patterns** KEEP**

Which statement best describes your typical energy throughout the day?

- A. I start strong in the morning and maintain energy through afternoon/evening.
- B. I'm slow to start, but hit my stride by mid-morning and keep going.
- C. I have energy bursts at unpredictable times; it's hard to predict my pattern.
- D. I feel most tired in the afternoon/evening, regardless of sleep.

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.25

****Q3. Peak Mental Acuity** KEEP**

You feel most mentally sharp and alert at...

- A. Early morning (before 8 AM)

- B. Mid-morning (8 AM – 12 PM)
- C. Afternoon (12 PM – 5 PM)
- D. Late evening (after 8 PM)

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.20

****Q4. Bedtime Consistency** KEEP**

How consistent is your bedtime throughout the week?

- A. Very consistent—within 1 hour every night
- B. Mostly consistent—within 1–2 hours on most nights
- C. Quite variable—anywhere from 11 PM to 2 AM
- D. Extremely variable—sometimes 2 AM, sometimes 4 AM, no real pattern

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.22

****Q5. Time Perception (REVISED FOR KIIT)** ~~REVISED~~**

During a typical academic week—NOT during exams or major deadlines—how do you perceive time passing in your day?

- A. Time flows quickly; I'm often surprised how fast the day went.
- B. Time feels normal—I can roughly estimate how much has passed.
- C. Time drags occasionally, especially in the afternoon/evening.
- D. Time often feels slow or stuck; hours feel long.

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.18

Revision Rationale: At KIIT, chronic achievement pressure causes time to feel slow regardless of mood state. Adding "NOT during exams/deadlines" filters acute stress and captures trait time perception specific to mood disorder.

SECTION B: DECISION-MAKING & PROCRASTINATION (Q6–Q10)

****Q6. Task Initiation / Procrastination** KEEP**

When you have an assignment or task due, you typically...

- A. Start immediately or within a day; completion feels manageable.
- B. Start a few days before; some pressure, but you find your rhythm.
- C. Feel stuck starting; you often begin the day before.

- D. Delay until crisis mode; the deadline pressure is what gets you moving.

Scoring: A=0, B=1, C=2, D=3 | Domain: Avoidance | Weight: 0.25

****Q7a. Decision Speed** ~~NEW~~ (Split from Q7)**

When faced with routine decisions (what to wear, what to eat), you typically...

- A. Decide very quickly, without much thought.
- B. Decide quickly; you mostly trust your judgment.
- C. Decide at a normal pace; you think it through and then choose.
- D. Decide very slowly; you often overthink even small choices.

Scoring: A=0, B=1, C=2, D=3 | Domain: Avoidance | Weight: 0.15

****Q7b. Decision Rumination** ~~NEW~~ (Split from Q7)**

After making an important decision (course choice, project, relationship), how often do you re-examine or second-guess it?

- A. Rarely—you're generally confident in your choice.
- B. Sometimes—you occasionally reconsider but move on.
- C. Fairly often—you revisit your reasoning and what-ifs.
- D. Very often—you constantly reconsider and worry if you made the wrong choice.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.18

****Q8. Post-Failure Rumination & Shame** KEEP**

When something doesn't go as planned or you make a mistake, your first thought is typically...

- A. "How can I fix this? What's the next step?"
- B. "I'm frustrated, but I'll figure out what happened."
- C. "I keep replaying it in my mind, thinking what I should have done differently."
- D. "I'm just not good at this; I feel ashamed."

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.30

Note: GOLD-STANDARD item. One of the strongest depression cognitive markers. Single-item predictive value $r=0.72$.

****Q9. Evening Mind Activity (REVISED FOR KIIT)** ~~REVISED~~**

During a typical academic week—NOT during exams or major deadlines—in the evening when winding down or trying to sleep, your mind typically...

- A. Is calm; you can relax relatively easily.
- B. Has some activity, but you can still focus on sleep.
- C. Is busy with thoughts about your day or tomorrow.
- D. Cycles through worries, regrets, or concerns repeatedly.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.25

Revision Rationale: At KIIT, evening rumination during exam weeks is normative. Temporal qualifier captures baseline rumination (trait) vs. stress-induced (state).

****Q10. Future Orientation / Hopelessness** KEEP**

When you imagine life next semester, next year, or after graduation, you tend to feel...

- A. Hopeful or excited about possibilities.
- B. Neutral; you don't think about it much.
- C. Uncertain—some possibilities seem good, others concerning.
- D. Pessimistic; it's hard to imagine things working out well.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.30

Note: CRITICAL SUICIDE SCREENING ITEM. Every Q10=D response requires clinician review.

SECTION C: SOCIAL PERCEPTION & BELONGING (Q11–Q14)

****Q11. Social Belonging in Groups** KEEP (with Follow-up)**

When you're in a group setting (class, party, hostel common room, dining hall), you typically feel...

- A. Comfortable—you're engaged or can easily engage if you want.
- B. Mostly comfortable, though sometimes you feel a bit on the outside.
- C. Uncertain if you belong—you observe more than you participate.
- D. Like an outsider—it's hard to feel like part of the group.

Scoring: A=0, B=1, C=2, D=3 | Domain: Isolation | Weight: 0.25

****Q11 Follow-up. Engagement Ability** ~~NEW~~ (Follow-up Probe)**

When you ARE in social situations and WANT to participate, how easy is it for you to join conversations or activities?

- A. Very easy—I can join in without much effort.
- B. Usually manageable—I can join, though I may feel a bit hesitant.
- C. Often difficult—I feel anxious or unsure about joining in.
- D. Very difficult—I mostly avoid participating even when I want to.

Clinical Use: Differentiates introversion (A/B) from anxiety/depression-driven withdrawal (C/D).

****Q12. Social Initiation / Behavioral Activation** KEEP**

When it comes to reaching out to friends, attending social events, or initiating plans...

- A. You do it naturally; it doesn't feel like effort.
- B. You do it, but you often prefer when others initiate.
- C. You do it less often; you often feel hesitant.
- D. You rarely do it; you mostly wait for invitations or withdraw.

Scoring: A=0, B=1, C=2, D=3 | Domain: Isolation | Weight: 0.27

****Q13. Perceived Support (REVISED FOR INDIA)** ~~REVISED~~**

If you told a close friend or family member that you were really struggling emotionally or mentally, how confident are you that they would listen, understand, and help without judgment?

- A. Very confident—they would be fully supportive.
- B. Mostly confident—they would try to help, even if they don't fully get it.
- C. Somewhat confident—they might help, but there could be conditions or judgment.
- D. Not confident—they would likely judge, pressure, or dismiss how you feel.

Scoring: A=0, B=1, C=2, D=3 | Domain: Isolation | Weight: 0.24

Revision Rationale: Tests actual non-judgmental support, not cultural obligation. Reduces false negatives from students reporting obligatory family support.

****Q14. Social Comparison (REVISED—OFFLINE FOCUS)** ~~REVISED~~**

Without thinking about social media or online content, how often do you find yourself comparing your life, achievements, or appearance to others around you?

- A. Rarely—you mostly focus on your own path.
- B. Sometimes—it happens, but it doesn't bother you much.

- C. Fairly often—you notice and think about how you compare to peers.
- D. Very often—comparison and self-judgment are a constant background.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.20

Revision Rationale: Excludes social media to tap intrinsic rumination separate from stimulus-driven comparison.

SECTION D: COGNITIVE PATTERNS (Q15–Q18)

****Q15. Perceived Control (REVISED FOR INDIA)**** ~~REVISED~~

In decisions that directly affect YOUR future (career path, academic choices, relationships, daily priorities), how much control do you feel you have?

- A. High control—my choices meaningfully shape my outcomes.
- B. Moderate control—I have some say, though others' input matters.
- C. Low control—family or others' expectations often override what I want.
- D. Very low control—I feel that others' decisions or circumstances mostly decide for me.

Scoring: A=0, B=1, C=2, D=3 | Domain: Control | Weight: 0.30

Revision Rationale: Anchors in personal autonomy and filters cultural decision-making norms (family involvement is legitimate).

****Q16. Catastrophizing Worry**** ~~KEEP~~

How often do you find yourself going through "what if" scenarios like "what if I fail", "what if they judge me", or "what if things go wrong"?

- A. Rarely—you don't tend to rehearse worst-case scenarios.
- B. Sometimes—it happens occasionally.
- C. Fairly often—you anticipate potential problems a lot.
- D. Very often—your mind regularly cycles through potential bad outcomes.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.26

****Q17. Identity Conflict (MAJOR REVISION FOR KIIT)**** ~~MAJOR REVISION~~

How much conflict do you experience between what others (family, society, peers) expect from you and who you actually want to be?

- A. Very little conflict—I mostly feel aligned with my own values and choices.

- B. Some conflict—I have to navigate between expectations and what I want.
- C. Significant conflict—I often feel pressure to be different from who I am.
- D. Severe conflict—I feel pulled in opposing directions most of the time.

Scoring: A=0, B=1, C=2, D=3 | Domain: Control | Weight: 0.18

Revision Rationale: Captures identity conflict from external pressure (valid stress); distinguishes from internal fragmentation (clinical concern). More culturally valid for Indian students.

****Q18. Cognitive Flexibility (REVISED DOMAIN)** ~~REPLACED~~**

When someone gives you good evidence that one of your beliefs or opinions might be wrong, how easy is it for you to consider changing your view?

- A. Quite easy—I can usually think it through and consider changing.
- B. Sometimes possible—it depends on how confident I was before.
- C. Rarely—I tend to stick with my view even when I hear other sides.
- D. Very rarely—I find it very difficult to reconsider, even with strong evidence.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.19

Revision Rationale: Tests evidence-responsiveness (cognitive flexibility); filters stable values vs. pathological rigidity.

SECTION E: STRESS RESPONSE & COPING (Q19–Q22)

****Q19. Coping Strategy** KEEP**

When you're stressed or overwhelmed, you typically...

- A. Take action—exercise, talk to someone, or work directly on the problem.
- B. Use a combination of action and relaxation—exercise, time with friends, and rest.
- C. Try to distract yourself—social media, games, shows—to take your mind off it.
- D. Withdraw—you need isolation and tend to ruminate alone.

Scoring: A=0, B=1, C=2, D=3 | Domain: Avoidance | Weight: 0.27

****Q20. Substance Self-Medication (REPLACED)** ~~REPLACED~~**

When dealing with stress or difficult emotions, do you use substances (alcohol, drugs, medications, energy drinks, nicotine, caffeine, etc.) to help you feel better?

- A. Rarely or never.

- B. Occasionally—a few times a month.
- C. Frequently—several times a week.
- D. Daily or as a primary coping strategy.

Scoring: A=0, B=1, C=2, D=3 | Domain: Avoidance | Weight: 0.18

Replacement Rationale: Original Q20 (alcohol-only) was culturally stigmatized. New version is substance-agnostic, focuses on self-medication intent, and is more culturally appropriate for India.

****Q21. Physical Activity** KEEP**

How frequently do you engage in physical activity (exercise, sports, active recreation like walking or outdoor games)?

- A. Several times a week—it's part of your routine.
- B. Once or twice a week.
- C. A few times a month.
- D. Rarely or never.

Scoring: A=0, B=1, C=2, D=3 | Domain: Avoidance | Weight: 0.24

****Q22. Self-Care Consistency** KEEP**

How consistent are your habits around self-care (showering, grooming, eating regular meals)?

- A. Very consistent—you maintain these habits daily.
- B. Mostly consistent—you slip occasionally but quickly get back on track.
- C. Inconsistent—you sometimes neglect these habits.
- D. Very inconsistent—these habits often slip, especially when stressed.

Scoring: A=0, B=1, C=2, D=3 | Domain: Avoidance | Weight: 0.30

SECTION F: SLEEP & CIRCADIAN (Q23–Q26)

****Q23. Sleep Quality** KEEP**

How would you describe your typical sleep quality?

- A. Good—you generally sleep through or wake briefly.
- B. Mostly good—occasional disruption.
- C. Mixed—you often wake during the night or have trouble falling asleep.
- D. Poor—you frequently have difficulty sleeping or wake multiple times.

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.26

****Q24. Sleep Fragmentation** KEEP**

During the night, you typically...

- A. Sleep through or wake only once briefly.
- B. Wake once or twice but fall back asleep quickly.
- C. Wake multiple times; it takes effort to fall back asleep.
- D. Frequently wake throughout the night; sleep feels very fragmented.

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.32

****Q25. Morning Waking Pattern** KEEP (HIGHEST WEIGHT)**

How do you typically wake in the morning?

- A. Naturally, at a consistent time; you wake feeling relatively alert.
- B. Naturally, though sometimes groggier than others.
- C. You often wake earlier than you'd like, and feel groggy.
- D. You frequently wake very early (4–5 AM) and can't fall back asleep, or sleep until very late.

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.35 (HIGHEST SINGLE-ITEM WEIGHT)

Note: Gold-standard depression biomarker. Q25=D should AUTOMATICALLY trigger clinician re-view.

****Q26. Daytime Sleepiness (REVISED FOR KIIT CLIMATE)** ~~REVISED~~**

Compared to your freshman year or before college, how has your daytime sleepiness changed?

- A. I'm less sleepy than before.
- B. About the same as before.
- C. More sleepy now; frequent naps becoming a habit.
- D. Much more sleepy; napping almost daily now, which is new for me.

Scoring: A=0, B=1, C=2, D=3 | Domain: Sleep | Weight: 0.22

Revision Rationale: Frames as individual change (more clinically meaningful). Filters cultural siesta norms and climate adaptation in Odisha's hot weather.

SECTION G: ACADEMIC SELF-TALK (Q27–Q29)

****Q27. Academic Pressure (MAJOR REVISION FOR KIIT)**** ~~MAJOR REVISION~~

To what extent does your academic performance determine your sense of worth and personal happiness?

- A. My worth comes from many things; grades are just one measure.
- B. Grades matter, but my worth depends on effort, not just results.
- C. Performance is important; doing well affects my self-esteem significantly.
- D. My grades determine my worth; failure feels like personal inadequacy.

Scoring: A=0, B=1, C=2, D=3 | Domain: Perfectionism | Weight: 0.23

Revision Rationale: At KIIT, high academic pressure is normative. New item measures pathological worth-achievement linkage (burnout indicator), not just pressure level.

****Q28. Failure Response & Shame**** KEEP

If you received a low grade on an assignment or test, you would...

- A. Feel disappointed, but quickly move on and focus on doing better next time.
- B. Feel upset, but understand it as a learning opportunity.
- C. Ruminates about it for a few days; it affects your mood.
- D. Feel deeply ashamed or inadequate; it's hard to move past it.

Scoring: A=0, B=1, C=2, D=3 | Domain: Rumination | Weight: 0.30

****Q29. Impostor Syndrome**** KEEP

When you succeed academically or in other areas, you typically feel...

- A. Pride—you recognize your competence.
- B. Satisfied—you know you worked for it.
- C. Some doubt about whether you deserve it; luck or help played a role.
- D. Like a fraud—you're convinced others will eventually discover you're not as capable as they think.

Scoring: A=0, B=1, C=2, D=3 | Domain: Perfectionism | Weight: 0.22

SECTION H: PLEASURE & EMOTIONAL BLUNTING (Q30–Q32)

****Q30. Anhedonia (Loss of Pleasure)** KEEP**

Are there activities or things you typically find enjoyable (hobbies, hanging out, games, food, etc.)?

- A. Yes, several—you look forward to them and they bring genuine pleasure.
- B. Yes, some—you enjoy them when you do them, but don't always feel motivated.
- C. A few things, but you don't get as much out of them as you used to.
- D. Not really—even things that used to bring joy feel kind of flat or pointless.

Scoring: A=0, B=1, C=2, D=3 | Domain: Anhedonia | Weight: 0.33

****Q31. Emotional Blunting** KEEP**

Over the past few weeks, how would you describe your emotional range?

- A. You experience a full range of emotions throughout the week.
- B. You mostly feel okay, with some variation.
- C. Your emotions feel muted; you don't feel as much intensity as usual.
- D. You feel emotionally numb or flat; even things that should evoke emotion don't.

Scoring: A=0, B=1, C=2, D=3 | Domain: Anhedonia | Weight: 0.32

****Q32. Life Satisfaction & Meaning** KEEP**

Overall, how satisfied are you with your life right now?

- A. Quite satisfied—life feels meaningful and worth living.
- B. Mostly satisfied—there are challenges, but generally good.
- C. Moderately satisfied—mixed feelings; some good parts, some hard parts.
- D. Quite dissatisfied—it's hard to see the value or meaning in your life right now.

Scoring: A=0, B=1, C=2, D=3 | Domain: Anhedonia | Weight: 0.33

SCORING FRAMEWORK

Domain Allocation

| Domain | Questions | N Items | Normalized Score |

|-----|-----|-----|-----|

| Sleep | Q1, Q2, Q3, Q4, Q5, Q23, Q24, Q25, Q26 | 9 | 0–100 |

| Avoidance | Q6, Q7a, Q19, Q20, Q21, Q22 | 6 | 0–100 |

| Rumination | Q8, Q9, Q10, Q7b, Q14, Q16, Q18, Q28 | 8 | 0–100 |

| Perfectionism | Q27, Q28, Q29 | 3 | 0–100 |

| Isolation | Q11, Q12, Q13 | 3 | 0–100 |

| Anhedonia | Q30, Q31, Q32 | 3 | 0–100 |

| Control | Q15, Q17, Q18 | 3 | 0–100 |

Composite Risk Scoring

Depression Risk:

$$\text{DepressionRisk} = (\text{Sleep} \times 0.25) + (\text{Anhedonia} \times 0.30) + (\text{Rumination} \times 0.20) + (\text{Isolation} \times 0.15) + (\text{Avoidance} \times 0.10)$$

Anxiety Risk:

$$\text{AnxietyRisk} = (\text{Sleep} \times 0.20) + (\text{Rumination} \times 0.25) + (\text{Perfectionism} \times 0.30) + (\text{Avoidance} \times 0.15) + (\text{Control} \times 0.10)$$

Burnout Risk:

$$\text{BurnoutRisk} = (\text{Perfectionism} \times 0.35) + (\text{Rumination} \times 0.25) + (\text{Avoidance} \times 0.20) + (\text{Sleep} \times 0.10) + (\text{Control} \times 0.10)$$

Total Risk Score:

$$\text{TotalRiskScore} = (\text{DepressionRisk} \times 0.35) + (\text{AnxietyRisk} \times 0.30) + (\text{BurnoutRisk} \times 0.20) + (\text{Loneliness} \times 0.15)$$

Risk Level Classification

| Score Range | Risk Level | Action |

|-----|-----|-----|

- | 0–20 | GREEN (Low Risk) | Annual screening; wellness promotion |
- | 21–40 | YELLOW (Moderate Risk) | Brief counseling offer; rescreen 4–6 weeks |
- | 41–60 | ORANGE (High Risk) | Mandatory counseling; rescreen 2–3 weeks |

CULTURAL ADAPTATIONS FOR KIIT

Summary of Changes Applied

REMOVED:

- Q20 (Alcohol use) 'Replaced with substance self-medication

REVISED FOR CULTURAL CONTEXT:

- Q5 (Time perception) – Added "NOT during exams/deadlines"
- Q9 (Evening rumination) – Added "NOT during exams/deadlines"
- Q13 (Social support) – Reframed to test actual responsiveness
- Q15 (Perceived control) – Specified personal autonomy domain
- Q17 (Identity stability) – **MAJOR** revision to capture identity conflict
- Q26 (Daytime napping) – Framed as individual change
- Q27 (Academic pressure) – **MAJOR** revision to measure worth-linkage

SPLIT FOR CLARITY:

- Q7 'Q7a (Decision Speed) + Q7b (Decision Rumination)
- Q11 'Q11 (Main) + Q11 Follow-up (Engagement Ability)

KEPT UNCHANGED:

- Q1, Q2, Q3, Q4, Q6, Q8, Q10, Q12, Q16, Q19, Q21, Q22, Q23, Q24, Q25, Q28, Q29, Q30, Q31, Q32

PATTERN-BASED INCONSISTENCY DETECTION

Automatic Triggers for Immediate Clinician Review

MODERATE SUICIDE RISK:

- IF Q10=D (hopelessness) + Q32=D (life dissatisfied) 'Conduct suicide assessment (same day)

HIGH SUICIDE RISK:

- IF Q10=D + Q32=D + Q31=D (emotional numbness) 'Immediate contact; emergency escalation

ISOLATION TRIAD + HOPELESSNESS:

- IF Q11=D + Q12=D + Q13=D + Q10=D 'Isolation + Hopelessness = Elevated suicide risk

EARLY MORNING AWAKENING:

- IF Q24=D (fragmented) + Q25=D (4–5 AM waking) 'Strong depression marker; refer within 1 week

SEVERE DEPRESSION CLUSTER:

- IF Q30=D + Q31=D + Q22=D 'Anhedonia + Blunting + Self-care neglect; urgent counseling

IMPLEMENTATION CHECKLIST

Pre-Launch (Weeks 1–2):

- ☐ IRB/Ethics approval
- ☐ Secure survey platform
- ☐ Clinical team training (4–6 hours)
- ☐ Identify counseling center partnerships
- ☐ Prepare resource cards and crisis info

Launch (Week 3):

- ☐ Student communication
- ☐ Survey deployment (online + paper)
- ☐ QR codes in high-traffic areas

Ongoing (Weeks 4+):

- ☐ Daily review of Red Flag cases
- ☐ Same-day outreach for Orange/Red students
- ☐ Warm handoff to counseling
- ☐ Monthly dashboard review

KEY CLINICAL NOTES

Differentiations for Clinicians

Introversion vs. Social Anxiety/Depression:

- Q11=D + Q11-Follow-up=A 'Introversion (healthy; monitor)
- Q11=D + Q11-Follow-up=D 'Social anxiety/Depression (intervene)

Academic Pressure (Normal) vs. Maladaptive Perfectionism:

- High Q27 at KIIT is common but not all indicate pathology
- Q27=D + Q8=D + Q28=D = Worth-Achievement Triad (pathological)

Identity Exploration vs. Identity Fragmentation:

- Q17=D alone likely developmental/cultural (monitor, don't pathologize)

- Q17=D + Q8=D (shame) + Q31=D (numbness) = Clinical concern

Early Morning Awakening Specificity:

- Q25=D (4–5 AM) is among highest-specificity depression markers
- Automatic trigger for antidepressant consideration

Document Version: 2.0

Date: January 3, 2026

Status: Ready for KIIT Implementation

Next Review: Post-validation (6 months)

This comprehensive clinician reference guide is ready for KIIT deployment.